

Policy on Safety of Patients, Families, Staff and Visitors

«Hospital Name»

Document control

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Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Engineering In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The organisation's environment and facilities operate to ensure the safety of patients, their families, staff and visitors.

1. Purpose

This policy sets out how «Hospital Name» keeps its environment and facilities safe for patients, families, staff and visitors. It covers:

- Controlling access to high-security areas
- Identifying and safely managing hazardous materials
- Implementing spill-response plans
- Conducting electrical safety audits
- Managing material not in use

This policy covers safety of patients, families, staff and visitors specifically. Related duties — like patient assessment, clinical care, medication management, patient rights, infection control, quality and safety monitoring, or hospital governance — are covered in the hospital's other policies, not repeated here.

Words marked like this are defaults. A blank marked _____ must be filled before issue.

2. Scope

This policy applies to «Hospital Name» staff involved in facilities, utilities, medical gases, and fire or non-fire emergencies, including:

- Engineering, biomedical, nursing and departmental leaders
- The Engineering In-Charge
- The Medical Superintendent
- Departmental leaders
- The Quality Coordinator

This policy covers safety of patients, families, staff and visitors specifically. Related duties are covered in the hospital's other policies, not repeated here, including:

- Patient assessment
- Clinical care
- Medication management
- Patient rights
- Infection control
- Quality and safety monitoring
- Hospital governance

3. Policy standards

«Hospital Name» defines extra-security areas and controls access for staff, patients and visitors. Hazardous materials are identified, documented and handled safely at every stage. Spill plans are implemented with floor-accessible summaries and kits. Electrical safety audits are conducted annually. Material not in use is systematically identified and disposed of.

Staff follow written guidance and keep the records listed in the traceability table.

4. Non-negotiable rules

1. Do not operate critical areas (operating theatre, ICUs including NICU, labour room, emergency) without a written security plan defining access for staff, patients and visitors; designated extra-security areas must have documented controls such as CCTV.
2. Do not start or continue using a hazardous material that has not been identified and documented, or for which a sorting, storage, handling, transport and disposal procedure does not exist.
3. Do not use a hazardous material in an area where the summarised Material Safety Data Sheet is not accessible to floor staff and the hazardous-materials spill kit is not reachable.
4. Do not condemn or dispose of material not in use without following the written identification-and-disposal procedure.
5. Do not allow a calendar year to pass without a completed electrical safety audit of the facility with documented actions.
6. Do not bypass the stop-work authority in section 6 when the trigger conditions are met.

7. Staff who see a FMS.3 rule broken report it the same shift to the Engineering In-Charge or the Medical Superintendent.

5. What we do

5.1 Operational planning identifies areas which need to have extra security and describes access to different areas in the hospital by staff, patients, and visitors.

Operational planning identifies extra-security areas and describes access for staff, patients and visitors (at a minimum operating theatre, intensive-care units including neonatal if in scope, labour room and emergency). Vulnerable spots (dark areas, long corridors, critical-area doors) have a defined control such as closed-circuit television.

Written security guidance exists.

5.2 Patient safety aspects in terms of structural safety of hospitals, especially of critical areas are considered while planning, design and construction of new hospitals and re-planning, assessment and retrofitting of existing hospitals.

When this hospital plans, designs or constructs a new building, or re-plans or retrofits an existing one, patient-safety structural aspects of critical areas are considered. Indian Seismic Code IS 1893 (Part 1), latest version, is the minimum structural reference named in the Guidebook.

The Engineering In-Charge files what was applied. This element is evidenced on actual building work, not by a policy sentence alone.

5.3 The organisation conducts electrical safety audits for the facility

The organisation conducts electrical safety audits of the facility at least once a year to reduce risk to people and property and to prevent fire from short-circuiting. National Electrical Code of India 2023 is a reference. Thermal imaging may be used.

The Engineering In-Charge holds the last audit and actions.

5.4 There is a procedure which addresses the identification and disposal of material(s) not in use in the organisation.

There is a written procedure for identifying and disposing of material not in use (non-functioning items, excess stock, general scrap). Condemnation records sit with the Engineering In-Charge.

5.5 Hazardous materials are identified and used safely within the organisation.

Hazardous materials used here are identified and used safely: sorting, storage, handling, transport and disposal, using Material Safety Data Sheets. Common examples the Guidebook names include chemicals, blood and cultures, mercury, nuclear isotopes, medical gases, liquefied petroleum gas, steam and ethylene oxide.

Using an unidentified hazardous material is a stop-work trigger (section 6).

5.6 The plan for managing spills of hazardous materials is implemented

The spill plan for hazardous materials is implemented: a summarised Material Safety Data Sheet the floor can read, a hazardous-materials kit where those materials are stored, and trained handlers.

A kit in a locked office the floor cannot reach does not satisfy this element.

6. Stop-work authority

Do not use a hazardous material that has not been identified, or for which the spill plan is not implemented (no Material Safety Data Sheet path, no kit, or staff not trained for that material).

Stop-work applies to using that material. Immediate life-saving care that already depends on a stocked clinical product continues while the spill path is restored.

The person who stops tells the Engineering In-Charge and the Medical Superintendent the same shift. Refusing an unidentified hazardous material is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that FMS.3 is resourced and followed.
Engineering In-Charge	Owns day-to-day implementation and records for this standard.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.
departmental leaders	Run the department-level duties this standard names.

8. Quality monitoring (RCA → CAPA)

The Quality Coordinator audits this policy quarterly. The audit reviews:

- Records for a sample of this standard's objective elements, checked against the What-we-do steps.
- Documentary evidence is on file for each asterisked objective element in the sample.
- CORE objective elements show no critical gaps in the sample.
- Stop-work events (if any) are logged with outcome.

Root-cause analysis is required when a gap found in this audit remains open beyond 90 days.

This policy is reviewed annually, and sooner after a related facility change, utility failure, equipment recall or fire-plan change.

9. Training and staff acknowledgement

Staff covered by this policy are trained at induction and once a year after that. Training covers the What-we-do steps, non-negotiables and stop-work (if present).

Staff acknowledgement

I have read the Policy on Safety of Patients, Families, Staff and Visitors of «Hospital Name». I will follow the processes described.

Name: _____ Designation: _____

Department / floor: _____ Date: _____

Signature: _____

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

10. Distribution

This policy is distributed to:

- Medical Superintendent
- Engineering In-Charge
- Quality Coordinator
- departmental leaders
- staff covered by FMS.3

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
CORE	Core objective element (NABH)
FMS	Facility Management and Safety (NABH Hospitals 6th Edition chapter)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element

12. Traceability to NABH HCO Full Accreditation 6th Edition FMS.3

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where	Responsible
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			addressed	
FMS.3.a	Commitment*	Operational planning identifies areas which need to have extra security and describes access to different areas in the hospital by staff, patients, and visitors.	Section 3; 5.1	Engineering In-Charge
FMS.3.b	Excellence	Patient safety aspects in terms of structural safety of hospitals, especially of critical areas are considered while planning, design and construction of new hospitals and re-planning, assessment and retrofitting of existing hospitals.	Section 3; 5.2	Engineering In-Charge
FMS.3.c	Commitment	The organisation conducts electrical safety audits for the facility.	Section 3; 5.3	Engineering In-Charge
FMS.3.d	Commitment*	There is a procedure which addresses the identification and disposal of material(s) not in use in the organisation.	Section 3; 5.4	Engineering In-Charge
FMS.3.e	CORE*	Hazardous materials are identified and used safely within the	Section 3; 5.5; Section 6 Stop-work	Engineering In-Charge

		organisation.		
FMS.3.f	Commitment*	The plan for managing spills of hazardous materials is implemented.	Section 3; 5.6	Engineering In-Charge

13. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

FMS.3.a — Operational planning identifies areas which need to have extra security and describes access to different areas in the hospital by staff, patients, and visitors.

- Written security operational plan naming extra-security areas (operating theatre, ICU/NICU, labour room, emergency) and access rules for staff, patients and visitors.
- Vulnerable-spot control record — closed-circuit television coverage or equivalent for dark areas and long corridors.
- Review or update record of the security plan.

FMS.3.b — Patient safety aspects in terms of structural safety of hospitals, especially of critical areas are considered while planning, design and construction of new hospitals and re-planning, assessment and retrofitting of existing hospitals.

- Structural-safety design reference actually applied (for example Indian Seismic Code IS 1893 Part 1) for the specific building project.
- Engineering sign-off record for that construction, re-planning or retrofit project.
- Evidence of what was applied to real building work, not a policy statement alone.

FMS.3.c — The organisation conducts electrical safety audits for the facility.

- Completed electrical safety audit report, at least annually.
- Action-taken record against audit findings.
- Reference to the National Electrical Code or thermal-imaging method used.

FMS.3.d — There is a procedure which addresses the identification and disposal of material(s) not in use in the organisation.

- Written procedure for identifying and disposing of material not in use.
- Condemnation register listing items, dates and disposal method.
- Engineering In-Charge sign-off on the register.

FMS.3.e — Hazardous materials are identified and used safely within the organisation.

- Current hazardous-materials inventory (for example chemicals, blood/cultures, mercury, isotopes, medical gases, LPG, steam, ethylene oxide).
- Material Safety Data Sheet on file for each identified material.
- Handling, storage, transport and disposal record for a sample of materials.

FMS.3.f — The plan for managing spills of hazardous materials is implemented.

- Summarised Material Safety Data Sheet accessible on the floor where the material is stored.
- Hazardous-materials spill-kit inventory and location log.
- Training record for staff handling that material.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers (NABH), Accreditation Standards for Hospitals, 6th Edition (January 2025) — Facility Management and Safety, standard FMS.3.
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — FMS.3 interpretations.
- Bureau of Indian Standards. IS 1893 (Part 1) — Criteria for Earthquake Resistant Design of Structures, latest version. Referenced as the minimum structural-safety reference for construction, re-planning and retrofitting of critical areas.
- Bureau of Indian Standards. National Electrical Code of India, 2023. Referenced for electrical safety audits of the facility.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

«Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed. The clinical and technical content reflects recognised national and international guidance current at the date of preparation.

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